Erlotinib monotherapy

Which treatment this guide covers NCCP 00219a is first-line EGFR-mutated advanced non-small-cell lung cancer (NSCLC); 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Erlotinib alone · daily oral treatment

Active medicines: erlotinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Keep the prescribed anti-diarrhoea plan available from the first dose. Report loose stools early and keep drinking if able. Persistent or severe diarrhoea, weakness, inability to drink or much less urine requires urgent advice. The team decides whether to hold treatment and when it is safe to restart.
- Use the agreed skin, nail and mouth-care plan. Report rash, painful nails or mouth sores early. Emollients and sun protection may help; the team decides whether creams, antibiotics or other treatment are needed. Blistering or peeling skin needs urgent assessment.
- Erlotinib has minimal-to-low oral emetic risk. Agree any as-needed anti-sickness medicine with the team. Routine infusion premedication is not required.
- Give pharmacy all medicines and supplements. Strong CYP3A4 inducers or inhibitors, grapefruit and medicines for stomach acid can change exposure. Avoid proton pump inhibitor combinations unless the specialist resolves the plan. Necessary antacids require source-specific separation; ask for written timing. Warfarin needs extra blood-clotting checks. Smoking reduces exposure; get cessation support and do not change your cancer dose yourself.
- Avoid pregnancy. Use effective contraception during treatment and for at least 2 weeks after the final dose. Do not breastfeed during treatment or for at least 2 weeks after the final dose. Discuss fertility and future pregnancy plans before starting.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00219 v6 · p3 · NCCP V6 oral minimal-low: individual as-required antiemetics; NCIS V8 is parenteral and does not prescribe an oral course

Sources checked 2026-10-29 · SACTCheck v0.84.26 · Regimen-level drafts; prescriber and oncology-pharmacy verification required